

Seattle Gender Wellness Center

1420 5th Ave, Suite 3100 · Seattle, WA 98101 · (206) 555-0192

PATIENT

Rivera, Alex
DOB: 08/12/1995 (age 28) · she/her
MRN: GWC-2023-00447

VISIT

Date: September 10, 2023
Provider: Dr. Sarah Chen, MD — Endocrinology
Type: Follow-Up · HRT Monitoring (8 months on therapy)

& **ALLERGIES: Penicillin (rash/urticaria) · Sulfonamides (hives, facial swelling)**

SUBJECTIVE

Alex returns for HRT follow-up at 8 months. She reports good tolerance to her current regimen. She notes progressive breast development, softening of skin, and reduction in body hair growth. She reports increased fatigue over the past 6 weeks, which she attributes to work stress and irregular sleep. She denies chest pain, palpitations, shortness of breath, or leg swelling. She reports mild mood improvement overall and no depressive episodes.

OBJECTIVE — VITAL SIGNS

BP: 118/76 mmHg HR: 74 bpm Weight: 162 lbs BMI: 24.6

OBJECTIVE — TODAY'S LABORATORY RESULTS

TEST	RESULT	UNIT	FLAG
Estradiol (E2)	178	pg/mL	' in target
Testosterone, Total	55	ng/dL	H — bor derline
Prolactin	8.5	ng/mL	normal
Hemoglobin	13.9	g/dL	!“ expected
Potassium	4.8	mEq/L	normal, trending up
ALT	38	U/L	upper normal — monitor
AST	27	U/L	normal
Creatinine	0.88	mg/dL	normal
Vitamin D, 25-OH	22	ng/mL	L — deficient
Cholesterol, Total	198	mg/dL	borderlin e

ASSESSMENT & PLAN

1. Hormone Therapy — Progressing Well, Dose Adjustment

Estradiol 178 pg/mL — within therapeutic target range (100–300 pg/mL). Good response.
Testosterone 55 ng/dL — borderline above target (<50 ng/dL). Increasing estradiol dose to further suppress.
CHANGE: Increase Estradiol valerate to 3 mg sublingual twice daily.
Continue Spironolactone 100 mg daily.
Recheck estradiol and testosterone in 4 months (January 2024).

2. Vitamin D Deficiency — New Finding

25-OH Vitamin D: 22 ng/mL — below normal (<30 ng/mL is deficient; target 40–60 ng/mL).
This is unrelated to hormone therapy but may be contributing to fatigue.
START: Vitamin D3 2,000 IU daily. Recheck in 4 months.

3. Liver Enzymes — Monitoring Required

ALT 38 U/L — at upper limit of normal. This is a recognized effect of sublingual estradiol hepatic first-pass metabolism.
If ALT exceeds 56 U/L at next visit, consider switching to transdermal or intramuscular route.
Continue monitoring. Patient advised to minimize alcohol intake.

4. Potassium — Trending Upward on Spironolactone

Potassium 4.8 mEq/L — within normal range (3.5–5.1) but elevated from 4.1 baseline.

This is an expected pharmacologic effect of spironolactone (potassium-sparing diuretic).

Counsel patient to moderate dietary potassium (limit bananas, avocado, oranges in excess).

Recheck potassium at January labs. If >5.2, consider dose reduction of spironolactone.

5. Fatigue — Likely Multifactorial

Most likely related to Vitamin D deficiency and hormone fluctuations during dose adjustment.

Consider CBC with iron studies at January visit if fatigue persists after Vitamin D supplementation.

Hemoglobin decreasing as expected on feminizing HRT (from 14.8 !' 13.9 g/dL) — not anemia.

Follow-up: 4 months (January 2024)

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Electronically signed by: Dr. Sarah Chen, MD

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